

Perineal problems In a vaginal delivery, the perineum, the area between the vagina and anus, may stretch and subsequently feel sore. If you had a tear that needed stitches or an episiotomy, this can be especially painful. If, after stitches, the area becomes red and swollen or develop new pain, you may have an infection.

What to do Warm baths are soothing, as are chilled witch hazel pads. Pouring warm water over stitches when urinating eases stinging, and Kegel exercises can help with healing. Your doctor may prescribe prescription medications or suggest a stool softener or over-the-counter painkillers for mild to moderate pain. Cooling gel pads can reduce swelling and bruising, and a pillow or inflatable swimming tube can make sitting easier. If your pain won't go away or you have signs of infection (wound becomes hot, painful, and swollen or you notice pus) contact your doctor.

Secondary postpartum hemorrhage Excessive bleeding more than 24 hours and up to six weeks after the delivery occurs in 1 percent of women. The most common cause is retained tissues or bits of placenta, which may become infected. The bleeding may be accompanied by fever and abdominal pain. Infection is treated with antibiotics and retained tissues are removed under anesthesia.

Congenital problems in babies

Down syndrome affects 1 in 700 newborns. Babies with Down have some developmental delay, learning difficulties, and an increased risk of other congenital abnormalities. Down causes a number of typical features, such as floppy muscle tone at birth; upward slanting eyes; a single skin crease running across the palms; and a somewhat flat back of the head.

Causes Down is due to an extra chromosome 21. Increasing maternal age makes Down syndrome more likely.

What might be done Some parents who are told they are expecting a baby with Down syndrome choose to have a termination. Parents who continue with pregnancy usually find that their child has a good quality of life. Many enjoy a degree of independence when older; life expectancy is around 60 years.

Clubfoot describes the condition when a baby is born with one or both feet turning down and inward.

Causes doctors don't know the exact cause of clubfoot.

What might be done The most common treatment is the Ponseti method, in which the baby's foot is manipulated into the right position and a cast put on. The foot is repositioned and a new cast placed every week for several weeks. More severe cases may require surgery. The baby will wear casts and/or a brace for some time after surgery to prevent deformities from returning.

Hip dysplasia occurs when the hip socket is too shallow, making the hip easily dislocatable. If the condition is suspected or there is an increased risk, an ultrasound may be done.

Causes The risk is increased if there is a family history, if a baby is breech, or if a baby has [clubfoot](#).

What might be done Treatment involves placing baby in a Pavlik harness for a month or two to keep the hip in place and allow the sockets to develop properly. If problem goes undetected until the baby is walking, they'll walk with a limp and probably need surgery.

Cleft lip and palate is where the two halves of the face don't join properly, causing a gap in the upper lip and/or palate. This may be detected prenatally but isn't always so can be a shock initially. There's evidence that extra folic acid in pregnancy can reduce the risk.